



Tablets

Composition

Each tablet contains	
Paracetamol	500 mg
Pseudoephedrine HCl	30 mg
Chlorpheniramine Maleate	2 mg

Pharmacology and Pharmacokinetics

Febricol contains a combination of an antihistamine, decongestant and antipyretic analgesic. The analgesic and antipyretic actions of Paracetamol are similar to those of the salicylates. Paracetamol is only a weak inhibitor of prostaglandin synthesis. Analgesia is mediated peripherally and also centrally, whereas antipyresis is produced by a central action on the hypothalamic regulatory center.

Chlorpheniramine maleate is a histamine H₁-blockers and is generally effective in relatively low doses. It acts by occupying receptor sites on the effector cell, to the exclusion of histamine antagonist molecules, without inhibiting histamine production nor inactivates them. The antagonism is competitive and reversible.

Pseudoephedrine Hydrochloride is a sympathomimetic agent that stimulates alpha-adrenergic receptors in vascular smooth muscle. This causes vasoconstriction, resulting in an increase in nasal airway patency. It has action similar to ephedrine but less active as a central nervous stimulant. It has alpha and beta-adrenergic activities and causes bronchodilation.

All the active ingredients are readily and effectively absorbed from the gastrointestinal tract. Paracetamol is metabolised in the liver and excreted in the urine mainly as the glucuronide and sulphate conjugates. Chlorpheniramine maleate is also extensively metabolised in the liver and excreted mainly as metabolites in the urine. Pseudoephedrine is resistant to metabolism by monoamine oxidase and is largely excreted unchanged in the urine together with small amounts of its hepatic metabolite.

Indications

Febricol is indicated for the symptomatic relief of headache, muscular ache, fever, sneezing, nasal congestion, and itching and watery eyes, associated with flu, common colds and allergic rhinitis.

Dosage and Administration

Febricol is preferably taken after food. To be administered orally. Adults and children over 12 years : 1-2 tablets every 4-6 hours. Maximum 8 tablets per day.

Contraindications

Febricol is contraindicated in patients taking monoamine oxidase inhibitors or within two weeks after stopping such treatment, patients with severe hypertension or severe coronary artery disease and in patients with known hypersensitivity to any of the active constituent.

Warning and Precautions

This medicine may cause drowsiness. If affected, do not drive motor vehicles or operate machinery. Do not drink alcohol or take drugs that may produce drowsiness while taking this medicine.

Febricol should be used with caution in patients with renal dysfunction and patients on antidepressant therapy. Patients with hypertension, hyperthyroidism, heart disease or who are receiving monoamine oxidase inhibitors should not take Febricol. Patients with diabetes, glaucoma, or prostatic enlargement should also avoid Febricol, unless directed by doctor.

Allergy alert :

Paracetamol may cause severe skin reactions. Symptoms may include skin reddening, blisters or rash. These could be signs of a serious condition. If these reactions occur, stop use and seek medical assistance right away.

Serious skin reactions

Rarely, paracetamol may cause serious skin reactions such as acute generalised exanthematous pustulosis (AGEP), Stevens-Johnson Syndrome (SJS), and toxic epidermal necrolysis (TEN), which can be fatal. Patients should be informed about the signs of serious skin reactions, and use of the drug should be discontinued at the first appearance of skin rash or any other sign of hypersensitivity.

This preparation contains Paracetamol. Do not take any other Paracetamol containing medicines at the same time. There is potential for liver toxicity and severe liver damage. Do not take more than the recommended dose.

Use in Pregnancy and Lactation

As with any drug, use with caution in pregnancy and lactation.

Drug Interaction

Febricol may interact with other sympathomimetic agents such as decongestants, tricyclic antidepressants, appetite suppressants and amphetamine-like psychostimulants and monoamine oxidase inhibitors (MAOIs). It may also interact with bretylium, bethanidine, guanethidine, debrisoquine, methyldopa, α - and β -adrenergic blocking agents.

Overdosage and Treatment

Paracetamol in massive overdosage may cause hepatic toxicity in some patients. Early symptoms following a potentially hepatotoxic overdose may include: nausea, vomiting, diaphoresis and general malaise. Clinical and laboratory evidence of hepatic toxicity may not be apparent until 48 to 72 hours post-ingestion. In adults and adolescents, regardless of the quantity of paracetamol reported to have been ingested, administer acetylcysteine immediately if 24 hours or less have elapsed from the reported time of ingestion. Promptly initiate gastric decontamination of the stomach. A plasma paracetamol assay should be obtained as early as possible, but no sooner than four hours following ingestion. If plasma level falls above the lower treatment line on the paracetamol overdose nomogram, acetylcysteine therapy should be continued. Liver function studies should be obtained initially and repeated at 24-hour intervals.

Serious toxicity or fatalities are extremely infrequent in children, possibly due to differences in the way they metabolise paracetamol.

Symptoms of pseudoephedrine overdose consist more often of mild anxiety, tachycardia and/or hypertension. Symptoms usually appear within 4 to 8 hours of ingestion and are transient, usually requiring no treatment.

Chlorpheniramine toxicity should be treated as an antihistamine/anticholinergic overdose and is likely to be present within a few hours after ingestion.

Side Effects

Most common side effects are sedation, including lassitude, dizziness and incoordination. CNS stimulation may occur, include anxiety, restlessness, insomnia, nervousness, euphoria, irritability, confusion and tremors. Gastrointestinal disturbances such as nausea, vomiting, diarrhoea, or epigastric pain have occurred. Other side effects include dry mouth, thickened respiratory-tract secretions and tightness of the chest; blurred vision; urinary retention in males where prostatic enlargement is present and constipation. Transient bradycardia followed by tachycardia with palpitations and arrhythmias and hypertension may also occur.

Side effects of paracetamol are rare and usually mild, though haematological reactions have been reported. Cutaneous hypersensitivity reactions including skin rashes, angioedema, Stevens Johnson Syndrome/Toxic Epidermal Necrolysis have been reported.

Shelf-life

The expiry date is indicated on the packaging.

Presentation

Light green colour, round-shaped and flat bevelled edge tablet, embossed with logo and breakline on the reverse side. 20's and 100's in blister packs of 10's. Not all pack size(s) is available.

Storage

Store in a cool dry place below 30°C.

KEEP OUT OF REACH OF CHILDREN/JAUHI DARI KANAK-KANAK

For further information, please consult your pharmacist or physician.

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Manufactured and Marketed by

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